

Intranasal neural protocol

NOVA-E2 2 cc via intranasal catheter · NOVA-M1 1-2 cc deltoid IM

Intranasal administration through the olfactory and trigeminal pathways bypasses the blood-brain barrier. Exosomes may support reduction of neuroinflammation, neuronal repair signaling, and modulation.

TOTAL VOLUME**2 cc**

1 cc per nostril

INSTILL RATE**10-20 sec**

slow, per nostril

HEAD POSITION**30-45°**

back-tilted, hold 8-10 min

DELTOID IM**1-2 cc**

NOVA-M1

Preparation

- Confirm no active sinus infection, severe congestion, or recent nasal surgery
- No nose blowing for 30 minutes before the procedure
- Review contraindications (pregnancy, active infection, recent chemotherapy)
- Obtain informed consent
- Seated in a treatment chair, head tilted back 30-45° to expose the olfactory cleft
- Draw 2 cc into sterile syringes (1 cc per nostril); remove air bubbles
- Attach syringes to the catheter per manufacturer instructions

POST-PROCEDURE INSTRUCTIONS

- No nose blowing for at least 60 minutes
- Avoid bending forward for 20-30 minutes
- Hydrate normally; avoid alcohol the same day
- Mild throat drainage sensation is normal

SAFETY

- Observe 5-10 minutes post-procedure
- Monitor for sinus discomfort or infection
- Consider ENT clearance for severe deviated septum

Administration sequence

- 1 Left nostril.** Insert catheter into position; instill 1 cc slowly over 10-20 s. Maintain head-back position and hold 8-10 minutes. Instruct the patient not to sniff heavily or swallow forcefully.
- 2 10-minute interval.** Keep the head back; wait 10 minutes before the second nostril.
- 3 Right nostril.** Instill 1 cc slowly over 10-20 s; maintain head-back position another 8-10 minutes.
- 4 Deltoid IM.** Administer 1-2 cc NOVA-M1 intramuscularly.

FOLLOW-UP**2 weeks**

First review

6 weeks

Second review

4-6 wks

Repeat interval by severity

EXPECTED RESPONSES

- Reduced hyperarousal, intrusive thoughts, and nightmares
- Improved emotional regulation and cognitive clarity
- Typical onset 3-7 days; fuller change at 8-12 weeks

CHART DOCUMENTATION · EVALUATION VISIT

PATIENT NAME	DATE OF BIRTH	PATIENT ID / MRN	DATE OF VISIT
EVALUATING PHYSICIAN	INITIAL · FOLLOW-UP VISIT TYPE — CIRCLE	REFERRING PROVIDER, IF ANY	TIME IN / OUT

S SUBJECTIVE

Neurologic symptoms per history —

CHIEF COMPLAINT	ONSET / DURATION	SEVERITY — VAS 0-10
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QUALITY / RADIATION	AGGRAVATING FACTORS	RELIEVING FACTORS
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Current neurology care · medications reviewed · prior therapies — list

PRIOR TREATMENT — CIRCLE ALL TRIED · ADD DATES

cognition · energy · mood · sleep · daily function FUNCTIONAL IMPACT	fever · chills · weight loss · neuro changes — note positives PERTINENT ROS
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MEDICATIONS — INCL. ANTICOAGULANTS, IMMUNOSUPPRESSANTS	ALLERGIES · RELEVANT PMH — CANCER, INFECTION, PREGNANCY
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O OBJECTIVE

BP	HR	TEMP	WEIGHT	GENERAL APPEARANCE
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General and mental status

Cranial nerves

Motor and coordination

Gait and balance

Nasal passages — patency, no active sinusitis

IMAGING / STUDIES REVIEWED — MODALITY, DATE, FINDINGS	BASELINE — Cog screen	BASELINE — Diary issued	BASELINE — Baseline sx
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A ASSESSMENT

Physician-directed neurologic support (per treatment plan)

WORKING DIAGNOSIS — CONFIRM OR AMEND	ADDITIONAL DIAGNOSES / CODES
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☐ Appropriate candidate — indications met, contraindications absent	☐ Alternatives discussed, including conservative care and doing nothing
☐ Non-FDA-approved status explained; no outcome promised	☐ Patient questions answered; understanding confirmed

P PLAN

Proposed treatment. Intranasal NOVA-E2 1-2 cc in divided aliquots per naris plus NOVA-M1 2 cc deltoid IM, 23 G. Observation 15-30 min. Symptom diary issued and explained.

☐ Informed consent (CN-PC) reviewed and signed	☐ Procedure performed today — see CN-OP-INN · or scheduled (date right)
☐ Fees, non-coverage by insurance, and refund policy reviewed	☐ Follow-up: review at 4-8 wk with symptom diary

PHYSICIAN SIGNATURE	PRINTED NAME & LICENSE NUMBER	PROCEDURE DATE, IF SCHEDULED
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PATIENT NAME

DATE OF BIRTH

PATIENT ID / MRN

DATE OF PROCEDURE

PERFORMING PHYSICIAN

Physician-directed neurologic support (per treatment...)

PRE-PROCEDURE DIAGNOSIS — CONFIRM OR AMEND

START TIME

ASSISTING

STAFF

PRE-PROCEDURE CHECKLIST

Check each box before the first draw

- | | |
|--|---|
| ☐ Informed consent (CN-PC) signed and on file; SOAP note (CN-SN-INN) complete | ☐ Screening reviewed — no contraindications present |
| ☐ Time-out performed — correct patient, site, product, route | ☐ Five checks read aloud with second staff: ID · product & lot · expiry · volume & route · consent |
| ☐ Baseline outcome measures recorded: Symptom diary · cognitive screen · adverse events | ☐ Site marked and prepped — chlorhexidine / povidone-iodine (circle); chain-of-custody log initiated |

BP · HR · TEMP

PRE-PROCEDURE VITALS

agent · concentration · volume — write NONE i...

LOCAL ANESTHETIC

BP · HR · TEMP

POST-OBSERVATION VITALS

PRODUCTS ADMINISTERED

Divide intranasal volume between nares in small aliquots

PRODUCT & TARGET	PLANNED	LOT	EXPIRY	GIVEN
NOVA-E2 (exosome) Intranasal via atomizer or soft catheter, both nares	1-2 cc			
NOVA-M1 Deltoid IM	2 cc			

TECHNIQUE

Position: reclined, head tilted back

- Small aliquots per naris, alternating sides; keep patient reclined 10–15 minutes after the final aliquot.
- Deltoid IM component: aspirate before injecting, inject slowly.
- Nasal passages clear before starting; postpone if significant congestion or active sinus infection.

BOTH NARES + DELTOID L · R — circle deltoid side
SITE / SIDE**ATOMIZER · SOFT CATHETER — circle**
GUIDANCEn/a intranasal · 23 G IM
NEEDLE USEDtarget 15–30 min
OBSERVATION — MINUTES

NARRATIVE — FINDINGS, ASPIRATE, DEVIATIONS FROM PROTOCOL, PATIENT RESPONSE

POST-PROCEDURE & DISPOSITION

Tolerated: **WELL · WITH EVENTS — circle**Discharged: **STABLE · AMBULATORY · WITH ESCORT — circle**

- ☐
- Patient note CN-PN-INN reviewed and given at discharge

COMPLICATIONS / ADVERSE EVENTS — WRITE "NONE" IF NONE

Any adverse event also requires the adverse-event response steps in this book.

Follow-up plan. Repeat consideration at 4–8 weeks based on the symptom diary and clinical response.

FOLLOW-UP SCHEDULED FOR

PHYSICIAN SIGNATURE

PRINTED NAME & LICENSE NUMBER

DATE & END TIME

Today you received an exosome product given as a fine mist through the nose, plus one intramuscular cell injection in your upper arm. The products used are made from donated birth tissue and are not FDA-approved treatments. This page tells you what is normal in the days ahead, how to care for yourself, and when to call us.

DATE OF TREATMENT

SITE(S) TREATED

NEXT VISIT

WHAT TO EXPECT

Changes are gradual — do not judge the result in the first days

FIRST FEW HOURS

A drip at the back of the throat, an odd taste, mild congestion, or sneezing are all normal, along with soreness at the arm injection site.

FIRST WEEK

Nothing dramatic is expected. Keep your symptom diary daily — small notes beat big impressions.

WEEKS 4-8

We review your diary against baseline at follow-up and decide whether a repeat treatment makes sense.

DO

- Stay reclined at the clinic for the full observation period
- Keep the daily symptom diary we gave you
- Drink water normally through the day
- Take acetaminophen (Tylenol) for arm soreness if needed

AVOID

- Blowing your nose forcefully for 4–6 hours
- Nasal sprays for 24 hours unless prescribed
- Alcohol for 72 hours
- Anti-inflammatory pain relievers for 72 hours

CALL US RIGHT AWAY IF

For an emergency, call 911 first

- A nosebleed that does not stop with gentle pressure
- A severe headache or a stiff neck with fever
- Fever above 101 °F
- New confusion, or trouble breathing

CLINIC PHONE

AFTER-HOURS PHONE

NOTES FROM YOUR VISIT